

The competitive advantage

Why a well-being-first, compliance-native EHR wins a market legacy vendors structurally can't serve.

The market. Physician burnout is a measurable, multibillion-dollar drain on health systems — and the EHR is a leading cause. LumaChart is the hospital EHR built to reverse it, and to serve public health.

FOUR ADVANTAGES LEGACY CAN'T EASILY COPY

1 • Evidence on the record

Care is cited at the point of decision. Adoption alone never raised quality — **intensive decision-support use did** [PMID 19390094](#). Not a bolt-on AI toggle beside Epic.

2 • Compliance as a live engine

ONC Part 170 by design; governed AI [§170.315\(b\)\(11\)](#) under an FDA/PCCP posture; a 50-state + PR statutory-deadline engine; transparent pricing [CMS 45 CFR 180](#). The moat.

3 • Well-being first

Burnout is the disease it treats — the design constraint. Epic and Oracle Health are built around the revenue cycle; the note is a billing artifact.

4 • Low-cost, open, standards-native

A nationwide *and* low/middle-income lane [PMID 37991820](#), aligned to CMS's modular, interoperable Medicaid (MES) goals. Standards, not lock-in.

THE MOAT

- **Evidence + compliance + implementation method.** National EHRs fail on people and process, not software [PMID 33017724](#) — so our edge compounds exactly where legacy is weakest, and it's hard to retrofit onto a billing-first monolith.

BUSINESS MODEL & FUNDING

- **Non-dilutive first.** NIH SBIR (AHRQ → NLM) funds the R&D — no equity given up to start.
- **Open-core revenue.** The core is open and low-cost; revenue comes from hosting, support, certified builds, and implementation services — the recurring, defensible layer.

WHERE WE ARE — CANDIDLY

- **A working, evidence-cited prototype** — 66 peer-reviewed sources on the record, certification-by-design, and active engagement on the CMS MES IT-Standards RFI.
- **Not yet deployed or certified.** That maturity — a certified pilot at a real site — is precisely what the raise funds.